00 — Project Charter

Project title

Standardizing Social Needs Screening and Referral at a Mobile Clinic Serving People Experiencing Homelessness (Chantilly, VA)

Problem statement

The mobile clinic serves about 30 adults experiencing homelessness in western Fairfax County. Staff do not use a standard way to ask about social needs. Whether a patient is asked about food, transportation, safety, or documents depends on the clinician. Positive answers are not documented in a consistent place, and referral follow-up is not tracked. As a result, needs go unidentified, referrals are inconsistent, and the clinic cannot tell whether patients are connected to services.

Background

- Unmet health-related social needs (HRSNs) are associated with worse health outcomes and higher acute-care use. For people experiencing homelessness, several HRSNs often occur together.
- Standardized instruments exist, such as CMS AHC-HRSN and NACHC PRAPARE. Their value depends on a reliable workflow and a referral pathway the clinic can actually deliver (see 01).

SMART aim (global)

> By the end of the 12-week intervention period, **≥ 80 %** of adult patient visits will have an up-to-date (completed within 90 days) standardized social needs screen (baseline estimated < 30 %), and **≥ 90 %** of patients with a positive screen will be offered a referral for every positive domain at the same visit.

Secondary aim: **≥ 50 %** of accepted referrals will have a documented follow-up outcome within 30 days.

Targets are provisional. Replace the baseline estimate after the baseline chart audit (07).

Scope

In scope	Out of scope (for now)
Adults (≥ 18) seen at the mobile clinic	Pediatric screening
Domains: housing, food, transportation, utilities, interpersonal safety, health literacy, ID/documents	Behavioral health and substance-use screening (separate clinical workflow)
Staff training, screening workflow, referral offer, follow-up tracking	Building software (Phase 2 decision)
Fairfax County / Chantilly resources	Direct provision of services

Team (fill in names)

Role	Responsibility	Name
Project lead (student)	Planning, data analysis, write-up	
Faculty advisor / preceptor	Academic oversight, QI determination	
Clinic sponsor (medical director / manager)	Approval, removing barriers	
Screening champion (RN / MA / CHW)	Day-to-day workflow, peer coaching	
Referral/follow-up owner (CHW / case manager)	Warm handoffs, 30-day follow-up	
Patient/lived-experience advisor	Wording and acceptability review	

Timeline (default 16 weeks — adjust when decided)

Weeks	Phase
−4 to 0	Preparation: approvals, QI/IRB determination, resource verification, training
1–4	Baseline: retrospective/concurrent chart audit, no change in practice
5–8	PDSA cycle 1: paper screening at intake by one champion
9–12	PDSA cycle 2: spread to all staff + referral offer workflow
13–16	PDSA cycle 3: 30-day follow-up calls / closed-loop tracking
17+	Sustainment plan, final analysis, SQUIRE 2.0 write-up

Measures (summary — see 07)

- **Process:** % visits with up-to-date screen; % due screens completed; % positive screens offered referral.
- **Outcome:** % accepted referrals with confirmed connection within 30 days.
- **Balancing:** added visit time; patient decline rate; staff-reported burden.

Risks and mitigations

Risk	Mitigation
Screening for needs the clinic cannot address erodes trust	Verified resource directory before go-live; ask whether the patient wants help
Safety disclosures in a non-private mobile space	Ask safety items only when privacy is possible; script for deferral
Patients hard to reach for follow-up	Collect best contact method; follow up at the next clinic visit
Small sample (n≈30) limits statistical inference	Use run charts and run rules, not hypothesis tests
Staff turnover/volunteers	Brief job aid, champion model, training in onboarding

Approvals

- [ ] Clinic sponsor sign-off
- [ ] Academic program / faculty sign-off
- [ ] QI vs. research determination (08)